

High-Alert Medication Management

Policy Number	POL-PH-032	Owner / Department	Pharmacy
Effective Date	2023-05-20	Version	3.0
Last Revised	2026-02-25	Next Review	2028-02-25
Approved By	Pharmacy & Therapeutics Committee	Applies To	All staff who prescribe, dispense, administer, or stock medications identified as high-alert

1. Purpose

To define the identification, storage, prescribing, and administration controls for high-alert medications — drugs bearing a heightened risk of significant patient harm when used in error — consistent with the ISMP List of High-Alert Medications and CMS pharmaceutical-services requirements.

2. Definitions

- **High-alert medication** — a drug with a heightened risk of causing significant harm when used in error, even though mistakes may or may not be more common.
- **Independent double check (IDC)** — two qualified clinicians separately verify the drug, concentration, dose, rate, and patient without prompting each other.
- **Maximum concentration** — the highest approved concentration stocked for a given high-alert drug to limit the harm of a programming error.

3. Policy and Procedure

Pharmacy maintains and publishes the facility high-alert medication list, based on the ISMP list and local event history, and reviews it at least annually. High-alert products are labeled with an auxiliary “HIGH ALERT” indicator and are dispensed in standardized, ready-to-administer concentrations whenever possible.

1. Prescribe high-alert drugs through standardized order sets with built-in dose limits and alerts.
2. Perform an independent double check before administering the drugs designated as requiring IDC.
3. Use smart-pump drug libraries with hard limits for all high-alert IV infusions.
4. Remove concentrated electrolytes (e.g., potassium chloride, concentrated sodium chloride) from floor stock except in approved critical-care locations.
5. Segregate and distinctively label neuromuscular blocking agents, with warnings that they cause respiratory arrest/paralysis.

IMPORTANT — Concentrated potassium chloride and other concentrated electrolytes must never be stored in general nursing-unit floor stock. If a concentrated electrolyte is found in an unapproved location, secure it immediately, notify Pharmacy, and file a near-miss report within 24 hours.

4. Representative High-Alert Classes and Controls

Class / example	Primary control
Insulin (all forms)	IDC for IV infusions; separate U-100 from concentrated products.
Anticoagulants (heparin, DOACs)	Weight-based order sets; IDC for heparin infusions.
Opioids & parenteral sedatives	Smart-pump limits; PCA double check; controlled access.
Concentrated electrolytes (KCl)	Removed from floor stock; pharmacy-prepared only.
Neuromuscular blocking agents	Segregated storage; "paralyzing agent" warning labels.
Chemotherapy / cytotoxics	Independent verification of regimen, dose, and body surface area.

5. Severity & Escalation

Classify high-alert medication events on the POL-RM-013 scale; a high-alert error that reaches the patient is classified at least SEV-2. Grade the error by the NCC MERP index per POL-PS-003.

Severity	Example	Notify / timeframe
SEV-1	Paralytic given to a non-ventilated patient; fatal KCl bolus.	Provider, Nursing Supervisor & Risk Mgmt within 1 hour.
SEV-2	High-alert drug reaching the patient with temporary harm needing intervention.	Provider & Risk Mgmt within 4 hours.
SEV-3 / Near Miss	Missing IDC with no harm; wrong concentration intercepted by pharmacy.	Event report within 24 hours.

6. Documentation Requirements

- Drug, concentration, dose, and route, with the high-alert designation noted.
- Independent double check performed and both verifier names, where required.
- Smart-pump library selection and any limit override, with reason.
- Storage-location corrections for any misplaced high-alert product.

7. References

- ISMP List of High-Alert Medications in Acute Care Settings.
- ISMP Targeted Medication Safety Best Practices for Hospitals.
- CMS Conditions of Participation, 42 CFR 482.25 (pharmaceutical services).
- Related: POL-RM-013 (severity & reporting); POL-PS-003 (medication-error reporting); POL-PH-030, POL-PH-031, POL-PH-033.